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Acne

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O que é?

Acne é uma doença de pele que ocorre quando as glândulas secretoras de óleo (glândulas sebáceas) tornam-se inflamadas ou infectadas, provocando cravos, espinhas, cistos, caroços e cicatrizes.

A acne aparece na puberdade induzida pelo início da produção de hormônios femininos (estrógenos) e masculinos (andrógenos). Apesar da maioria dos casos de acne se resolver espontaneamente na segunda década da vida, há exceções e algumas pessoas continuam apresentando os sintomas durante a vida adulta, até cerca de 35 anos. A acne aparece com maior freqüência no rosto, peito e costas, onde o número de glândulas sebáceas é maior.

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O que é?

Desenvolve-se em pessoas com tendência hereditária; isto significa que um jovem, cujo pai e mãe tiveram acne, tem maior chance de apresentá-la. No entanto, você pode ser o primeiro a ter espinhas em sua família, assim como todos os irmãos ou somente um deles pode apresentar pele acneica. A acne pode ter graus variados, com maior ou menor inflamação. Ela sempre é mais grave quando apresenta cistos, caroços e muitas lesões.

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O que causa?

- Inflamação
- Vitamina B12 (alta ou baixa)
- Resistência insulina - IGF1
- Aumento DHT - SOP
- Ansiedade - depressão
- Intestino - disbiose
- Homocisteína alta - fígado
- Lácteos - whey - BCAA

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The Skin Microbiome: A New Actor in Inflammatory Acne

Brigitte Dréno[1,2] · Marie Ange Dagnelie[2] · Amir Khammari[1,2] · Stéphane Corvec[3,4]

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Abstract

Our understanding of the role of *Cutibacterium acnes* in the pathophysiology of acne has recently undergone a paradigm shift: rather than *C. acnes* hyperproliferation, it is the loss of balance between the different *C. acnes* phylotypes, together with a dysbiosis of the skin microbiome, which results in acne development. The loss of diversity of *C. acnes* phylotypes acts as a trigger for innate immune system activation, leading to cutaneous inflammation. A predominance of *C. acnes* phylotype IA_1 has been observed, with a more virulent profile in acne than in normal skin. Other bacteria, mainly *Staphylococcus epidermis*, are also implicated in acne. *S. epidermidis* and *C. acnes* interact and are critical for the regulation of skin homeostasis. Recent studies also showed that the gut microbiome is involved in acne, through interactions with the skin microbiome. As commonly used topical and systemic antibiotics induce cutaneous dysbiosis, our new understanding of acne pathophysiology has prompted a change in direction for acne treatment. In the future, the development of individualized acne therapies will allow targeting of the pathogenic strains, leaving the commensal strains intact. Such alternative treatments, involving modifications of the microbiome, will form the next generation of 'ecobiological' anti-inflammatory treatments.

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Review

Acne, Microbiome, and Probiotics: The Gut–Skin Axis

Pedro Sánchez-Pellicer [1], Laura Navarro-Moratalla [1], Eva Núñez-Delegido [1], Beatriz Ruzafa-Costas [1], Juan Agüera-Santos [1] and Vicente Navarro-López [1,2,*]

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Abstract: The objective of this narrative review was to check the influence of the human microbiota in the pathogenesis of acne and how the treatment with probiotics as adjuvant or alternative therapy affects the evolution of acne vulgaris. Acne is a chronic inflammatory skin disease involving the pilosebaceous units. The pathogenesis of acne is complex and multifactorial involving genetic, metabolic, and hormonal factors in which both skin and gut microbiota are implicated. Numerous studies have shown the bidirectionality between the intestinal microbiota and skin homeostasis, a communication mainly established by modifying the immune system. Increased data on the mechanisms of action regarding the relevance of *Cutibacterium acnes*, as well as the importance of the gut–skin axis, are becoming known. Diverse and varied in vitro studies have shown the potential beneficial effects of probiotics in this context. Clinical trials with both topical and oral probiotics are scarce, although they have shown positive results, especially with oral probiotics through the modulation of the intestinal microbiota, generating an anti-inflammatory response and restoring intestinal integrity, or through metabolic pathways involving insulin-like growth factor I (IGF-1). Given the aggressiveness of some standard acne treatments, probiotics should continue to be investigated as an alternative or adjuvant therapy.

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Citation: Sánchez-Pellicer, P.; Navarro-Moratalla, L.; Núñez-Delegido, E.; Ruzafa-Costas,

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Meta-Analysis > J Am Acad Dermatol. 2020 Aug;83(2):532-541.
doi: 10.1016/j.jaad.2020.02.040. Epub 2020 Feb 20.

Acne vulgaris and risk of depression and anxiety: A meta-analytic review

Danielle V Samuels [1], Robert Rosenthal [2], Rick Lin [3], Soham Chaudhari [3], Misaki N Natsuaki [2]

Affiliations + expand

PMID: 32088269 DOI: 10.1016/j.jaad.2020.02.040

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Abstract

Background: Several studies have shown an association of acne vulgaris with depression and anxiety, but a quantitative review has not yet been conducted.

Objective: We sought to conduct a systematic review and meta-analysis that elucidates the association of acne vulgaris with depression and anxiety.

Methods: A systematic review and meta-analysis of literature published before October 1, 2019 from the PubMed, PsycINFO, MEDLINE, and Cochrane databases was conducted. We used a metaanalytic approach to perform a random effects analysis comparing individuals with and without acne. Subgroup analyses between studies included age, study setting, and geographic region.

Results: Forty-two studies were included. We found a significant association of acne vulgaris with depression ($r = 0.22$ [95% confidence interval 0.17-0.26, $P < .00001$]) and anxiety ($r = 0.25$ [95% confidence interval 0.19-0.31, $P < .00001$]). Subgroup analyses and comparisons showed moderating influences based on factors including age, study setting, and geographic region.

Limitations: Inconsistency between publications regarding acne and outcome ascertainment, data reporting, and studies with no control group posed considerable barriers to synthesizing all available published literature.

Conclusions: Because of an increased risk for depression and anxiety, clinicians should pursue aggressive treatment of acne and consider psychiatric screening or referrals.

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Insulin resistance in the course of acne – literature review

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Abstract

Acne vulgaris is a very common skin disease being diagnosed in Westernized populations, however, its multifactorial etiopathogenesis still remains unclear. Recent research has demonstrated a possible linkage between acne and insulin resistance (IR), which is the topic of our review. In addition to an inappropriate diet, excessive androgen production or obesity, it is the IR which might be responsible for lack of efficacy of classical treatment strategy in acne. Interestingly, in all such cases an increased activity of mammalian target of rapamycin kinase complex 1 (mTORC1) has been detected. This observation might be considered as the basis of the possible role of metformin as an adjunct therapeutic modality for patients suffering from acne. The aim of our review is to present the possible etiological correlation between acne and insulin resistance, as well as metformin therapy, which might be highly useful in the treatment to resistant forms of acne.

Key words: acne vulgaris, resistant acne, insulin resistance, metformin.

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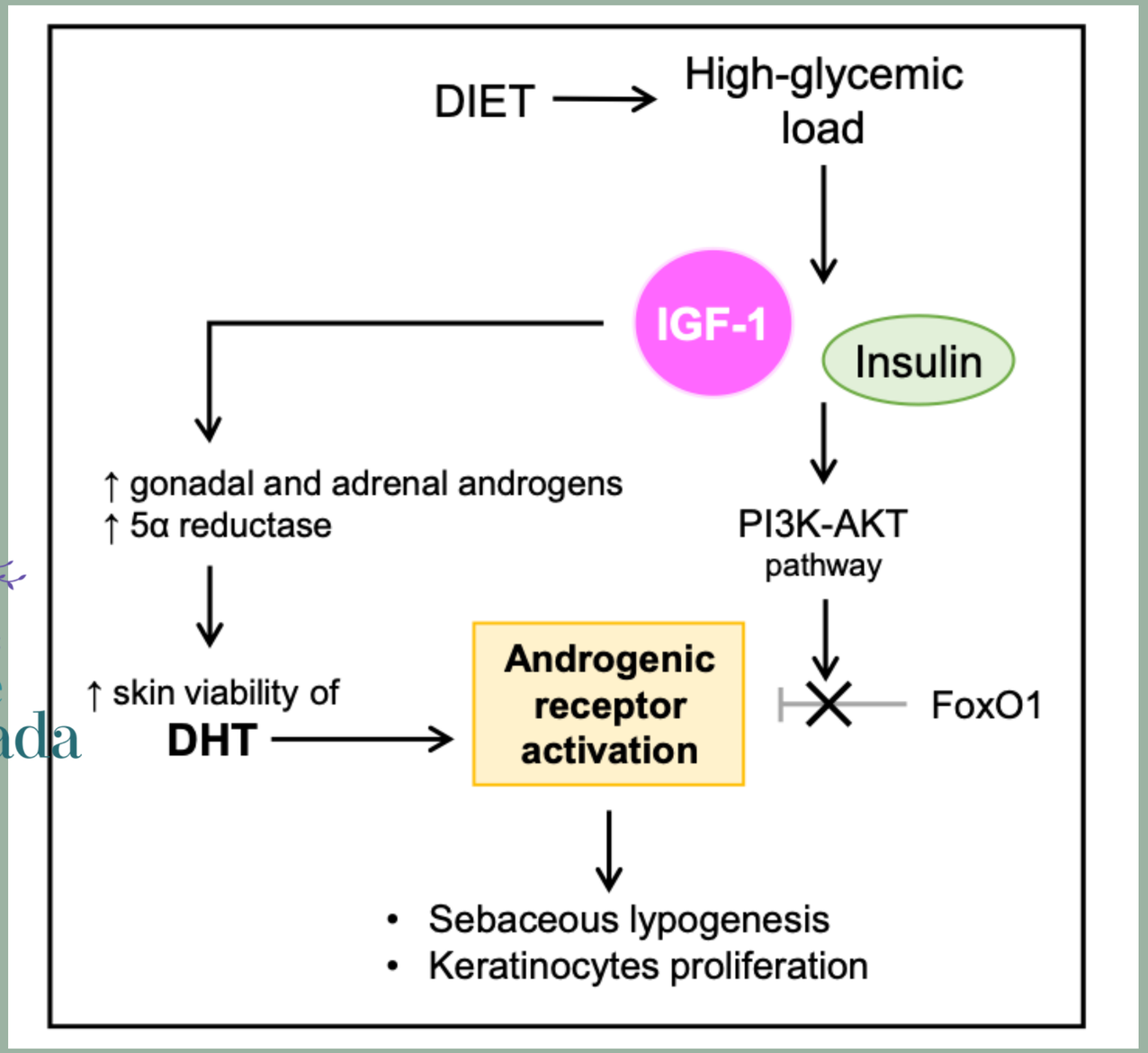

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Review > Clin Dermatol. 2004 Sep-Oct;22(5):419-28. doi: 10.1016/j.clindermatol.2004.03.010.

Acne: hormonal concepts and therapy

Diane Thiboutot [1]

Affiliations + expand

PMID: 15556729 DOI: 10.1016/j.clindermatol.2004.03.010

Abstract

Acne vulgaris is the most common skin condition observed in the medical community. Although we know that hormones are important in the development of acne, many questions remain unanswered regarding the mechanisms by which hormones exert their effects. Androgens such as dihydrotestosterone (DHT) and testosterone, the adrenal precursor dehydroepiandrosterone sulfate (DHEAS), estrogens such as estradiol, and other hormones, including growth hormone and insulin-like growth factors (IGFs), may be important in acne. It is not known whether these hormones are taken up from the serum by the sebaceous gland, whether they are produced locally within the gland, or whether a combination of these processes is involved. Finally, the cellular and molecular mechanisms by which these hormones exert their influence on the sebaceous gland have not been fully elucidated. Hormonal therapy is an option in women with acne not responding to conventional treatment or with signs of endocrine abnormalities.

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J Cosmet Dermatol. 2018 Jun;17(3):523-526. doi: 10.1111/jocd.12456. Epub 2017 Nov 21.

Serum homocysteine levels in acne patients

Hao Jiang [1], Changyi Li [1], Bin Wei [2], Qiang Wang [2], Jiang Zhong [1], Jianqi Lu [1]

Affiliations + expand

PMID: 29159884 DOI: 10.1111/jocd.12456

Abstract

Objective: This study was designed to investigate serum homocysteine (HCY) levels in acne patients.

Methods: Acne patients (n = 124) and healthy volunteers (n = 70), matched in terms of both age and sex, were enrolled. Serum HCY levels for all subjects were measured by a clinical laboratory.

Results: Serum HCY levels in male and female patients with severe and moderate acne were significantly higher than in the healthy control group ($P < .05$). The constituent ratio of male and female acne patients with HCY above the normal range (10 mmol/L) was significantly higher than the healthy control group. The severity of acne patients was positively correlated with serum homocysteine concentration, ($P < .01$).

Conclusion: Hyperhomocysteinemia may be an independent risk factor for acne vulgaris. Detection of serum HCY is important for acne patients.

Keywords: acne vulgaris; folate; homocysteine; vitamin B12; vitamin B6.

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Meta-Analysis > J Cosmet Dermatol. 2020 Mar;19(3):736-745. doi: 10.1111/jocd.13059.
Epub 2019 Jul 5.

Homocysteine, folic acid, and vitamin B12 levels in patients on isotretinoin therapy for acne vulgaris: A meta-analysis

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Affiliations + expand

PMID: 31276303 DOI: 10.1111/jocd.13059

Abstract

Background: Oral isotretinoin (Iso) is one of the most commonly used drugs for patients with moderate-to-severe acne; however, its use has been associated with several adverse effects. Some studies have suggested an association between Iso therapy and homocysteine (Hcy), folic acid, and vitamin B12 plasma levels.

Objective: To evaluate the changes in plasma Hcy, folic acid, and vitamin B12 levels during Iso therapy for acne using meta-analytic methods.

Methods: Five scientific databases (MEDLINE, EMBASE, Cochrane Library, SCOPUS, and Web of Science) were searched according to Preferred Reporting Items for Systematic Reviews and Meta-analyses (PRISMA) guidelines up to December 2018. A review of 734 publications identified 10 studies that assessed plasma levels of Hcy, folic acid, and vitamin B12 during Iso therapy in acne patients.

Results: A total of 10 studies consisting of 592 patients were included in the meta-analysis. Plasma Hcy levels were significantly increased after Iso therapy (weighted mean difference [WMD]: 2.99, 95% confidence interval [CI]: 1.78-4.20, I^2 = 86%), whereas folic acid levels were significantly decreased after Iso therapy (WMD: -1.03, 95% CI: -1.90 to -0.17, I^2 = 89%).

Conclusions: This meta-analysis found that Iso therapy was associated with changes in plasma levels of Hcy and folic acid in acne patients. However, further evaluation in controlled studies is needed to verify these results.

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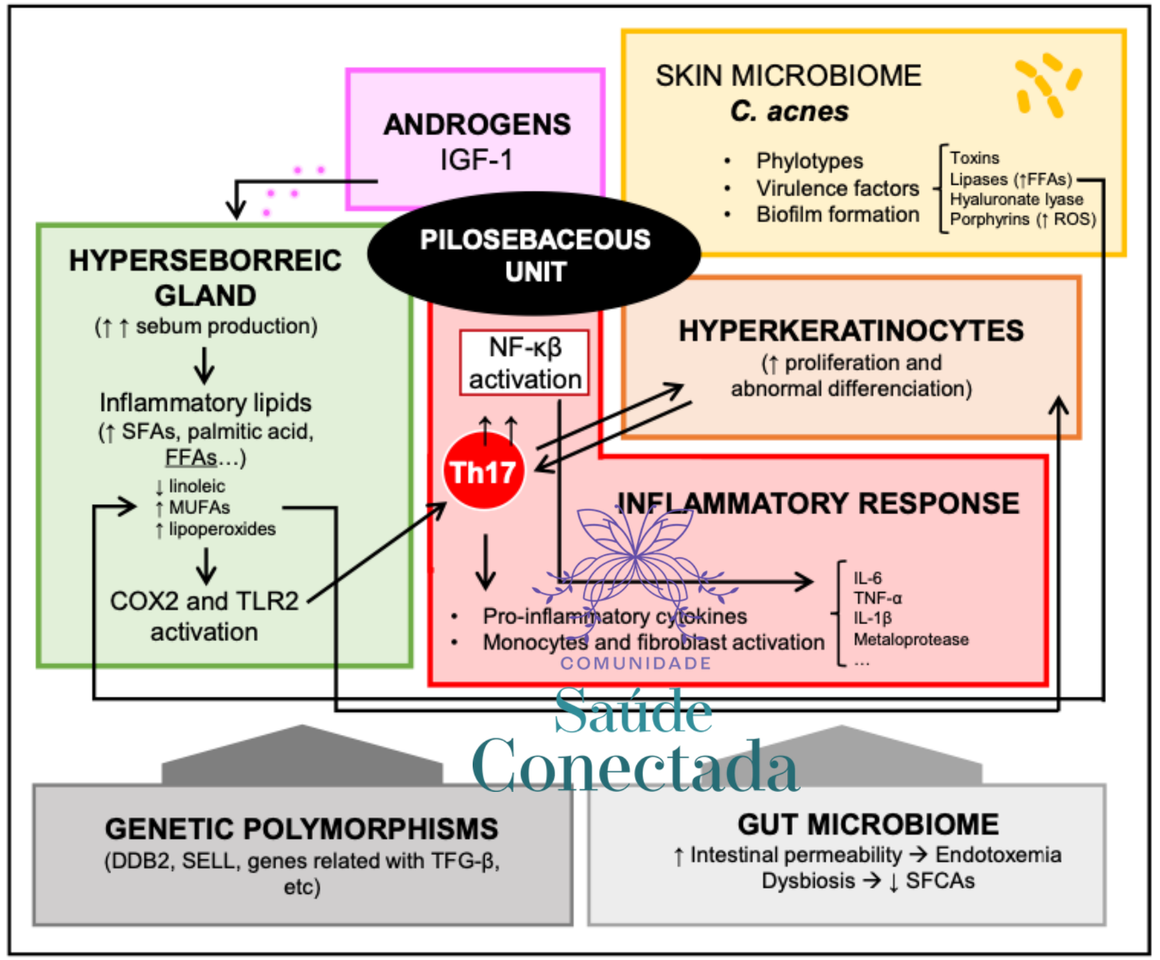

Figure 2. Pathogenic mechanisms of acne. IGF-1: insulin-like growth factor 1, FFAs: free fatty acids, ROS: reactive oxygen species, SFAs: saturated fatty acids, MUFAs: monounsaturated fatty acids, COX2: cyclooxygenase 2, TLR2: Toll-like receptor 2, Th17: Th17 cells, NF-κB: nuclear factor-kappa B, TGF-β: transforming growth factor receptor β, SCFAs: short-chain fatty acids.

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Review > Dermatol Online J. 2020 Aug 15;26(8):13030/qt9rp7t2p2.

Acne related to dietary supplements

Dina H Zamil, Ariadna Perez-Sanchez, Rajani Katta [1]

Affiliations + expand

PMID: 32941710

Free article

Abstract

Multiple prescription medications may cause or aggravate acne. A number of dietary supplements have also been linked to acne, including those containing vitamins B6/B12, iodine, and whey, as well as "muscle building supplements" that may be contaminated with anabolic-androgenic steroids (AAS). Acne linked to dietary supplements generally resolves following supplement discontinuation. Lesions associated with high-dose vitamin B6 and B12 supplements have been described as monomorphic and although pathogenesis is unknown, a number of hypotheses have been proposed. Iodine-related acne may be related to the use of kelp supplements and has been reported as monomorphic, inflammatory pustules on the face and upper trunk. Whey protein supplements, derived from milk and used for bodybuilding, are associated with papulonodular acne involving the trunk and sometimes the face. Finally, AAS-induced acne has been described as acne fulminans, acne conglobata, and acne papulopustulosa. With studies indicating that about half of US adults report using dietary supplements, it is important that dermatologists directly ask acne patients about their supplement use and educate them on the potential risks of even seemingly innocuous dietary supplements.

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Review > J Eur Acad Dermatol Venereol. 2018 Dec;32(12):2244-2253. doi: 10.1111/jdv.15204. Epub 2018 Sep 5.

The effect of milk consumption on acne: a meta-analysis of observational studies

R Dai [1 2], W Hua [2], W Chen [2], L Xiong [2], L Li [2]

Affiliations + expand

PMID: 30079512 DOI: 10.1111/jdv.15204

Abstract

Background: Acne is a common skin condition in developed countries with western diets. The effect of milk on acne has been highly controversial. To examine the association between milk consumption and acne risk, we conducted a meta-analysis of available data.

Materials and methods: We carried out comprehensive databases search of PubMed, Embase, Medline and Cochrane Library and identified four cohort studies and nine case-control or cross-sectional studies, including a total of 71 819 participants. We evaluated the pooled odds ratio (OR) with its 95% confidence interval (CI) using a random effects model. Subgroup analyses on acne severity, milk forms and milk intake levels were performed.

Results: Compared with non-consumers, the pooled OR was 1.16 (95% CI 1.09-1.24) for overall milk consumers in all included studies, and 1.17 (95% CI 1.10-1.24) in cohort studies and 1.16 (95% CI 1.09-1.24) in case-control or cross-sectional studies. Subgroup analysis on milk forms determined a stronger association in skim milk consumers (OR = 1.24, 95% CI 1.13-1.37) than in low-fat consumers (OR = 1.14, 95% CI 1.08-1.22) and full-fat consumers (OR = 1.13, 95% CI 1.05-1.21). The pooled OR was greater for high intake level of milk (OR = 1.12, 95% CI 1.01-1.24) than medium intake level of milk (OR = 1.08, 95% CI 1.00-1.17). A subset study of moderate-to-severe acne also found a positive association with milk consumption (OR 1.18, 95% CI 1.01-1.37), while no statistically significant association was found between mild acne risk and milk consumption (OR 1.14, 95% CI 0.86-1.51).

Conclusion: This meta-analysis provides evidence of a positive association between milk consumption and acne risk.

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> Rocz Panstw Zakl Hig. 2021;72(2):137-143. doi: 10.32394/rpzh.2021.0164.

Associations between diet and acne lesions

Magdalena Daszkiewicz [1]

Affiliations + expand

PMID: 34114770 DOI: 10.32394/rpzh.2021.0164

Free article

Abstract

Acne is one of the most common dermatological conditions affecting millions of people worldwide. It is assumed that the main etiological factors acne are considered to be the excessive production of sebum and hormones, and the colonisation of P. acnes bacteria. The relationship between diet and acne lesions is still the subject of debate and scientific criticism. On the one hand, studies suggest that an elimination diet is of no benefit in the treatment of acne, while other studies indicate that there is a close a close relationship between diet and the occurrence of acne lesions. Already in the 1970s, some scientists claimed that the course of acne could be modified by specific dietary components. Several studies have confirmed, among others, the role of a diet with a high glycemic index diet in acne exacerbation and suggested that a low glycemic index diet for several weeks may lead to a reduction in the number of acne lesions. The effect of milk consumption was also investigated in terms for its potential role in acne pathogenesis, including its ability to increase insulin levels. In the last decade, studies have also emerged to determine the effect of dark chocolate on the exacerbation of acne symptoms. To date, it has not been conclusively established which component of chocolate may have a potentially adverse effects on the course of acne. The article summaries the current knowledge on the relationship between diet and acne lesions, and discusses the results of recent studies on selected dietary components that may exacerbate acne.

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PRIMEIRO PASSO

Protocolo Desinflama Conecta

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REVIEW

Linking diet to acne metabolomics, inflammation, and comedogenesis: an update

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Abstract: Acne vulgaris, an epidemic inflammatory skin disease of adolescence, is closely related to Western diet. Three major food classes that promote acne are: 1) hyperglycemic carbohydrates, 2) milk and dairy products, 3) saturated fats including *trans*-fats and deficient ω-3 polyunsaturated fatty acids (PUFAs). Diet-induced insulin/insulin-like growth factor (IGF-1)-signaling is superimposed on elevated IGF-1 levels during puberty, thereby unmasking the impact of aberrant nutrigenomics on sebaceous gland homeostasis. Western diet provides abundant branched-chain amino acids (BCAAs), glutamine, and palmitic acid. Insulin and IGF-1 suppress the activity of the metabolic transcription factor forkhead box O1 (FoxO1). Insulin, IGF-1, BCAAs, glutamine, and palmitate activate the nutrient-sensitive kinase mechanistic target of rapamycin complex 1 (mTORC1), the key regulator of anabolism and lipogenesis. FoxO1 is a negative coregulator of androgen receptor, peroxisome proliferator-activated receptor-γ (PPARγ), liver X receptor-α, and sterol response element binding protein-1c (SREBP-1c), crucial transcription factors of sebaceous lipogenesis. mTORC1 stimulates the expression of PPARγ and SREBP-1c, promoting sebum production. SREBP-1c upregulates stearoyl-CoA- and Δ6-desaturase, enhancing the proportion of monounsaturated fatty acids in sebum triglycerides. Diet-mediated aberrations in sebum quantity (hyperseborrhea) and composition (dysseborrhea) promote *Propionibacterium acnes* overgrowth and biofilm formation with overexpression of the virulence factor triglyceride lipase increasing follicular levels of free palmitate and oleate. Free palmitate functions as a "danger signal," stimulating toll-like receptor-2-mediated inflammasome activation with interleukin-1β release, Th17 differentiation, and interleukin-17-mediated keratinocyte proliferation. Oleate stimulates *P. acnes* adhesion, keratinocyte proliferation, and comedogenesis via interleukin-1α release. Thus, diet-induced metabolomic alterations promote the visible sebofollicular inflammasomopathy acne vulgaris. Nutrition therapy of acne has to increase FoxO1 and to attenuate mTORC1/SREBP-1c signaling. Patients should balance total calorie uptake and restrict refined carbohydrates, milk, dairy protein supplements, saturated fats, and *trans*-fats. A paleolithic-like diet enriched in vegetables and fish is recommended. Plant-derived mTORC1 inhibitors and ω-3-PUFAs are promising dietary supplements supporting nutrition therapy of acne vulgaris.

Keywords: acne, comedogenesis, diet, inflammasome, metabolomics, quorum sensing

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Suplementos

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CSC

- Berberina
- Garcinia
- Green tea
- Vitex agnus castus
- Curcumina
- Ômega 3
- Vitamina D
- Probióticos

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Review > Skinmed. 2012 Nov-Dec;10(6):352-5.

Green tea in dermatology

Nader Pazyar [1], Amir Feily, Afshin Kazerouni

Affiliations + expand

PMID: 23346663

Abstract

The purpose of this brief review is to summarize all in vitro, in vivo, and controlled clinical trials on green tea preparations and their uses in dermatology. An extensive literature search was carried out to identify in vivo and in vitro studies as well as clinical trials. Twenty studies were assessed and the results suggest that oral administration of green tea can be effective in the scavenging of free radicals, cancer prevention, hair loss, and skin aging plus protection against the adverse effects associated with psoralen-UV-A therapy. Topical application of green tea extract should be potentially effective for atopic dermatitis, acne vulgaris, rosacea, androgenetic alopecia, hirsutism, keloids, genital warts, cutaneous leishmaniasis, and candidiosis. There are promising results with the use of green tea for several dermatologic conditions; however, the efficacy of oral and topical green tea has not always been confirmed.

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> Fitoterapia. 2007 Sep;78(6):401-8. doi: 10.1016/j.fitote.2007.02.019. Epub 2007 Jun 2.

Effect of Garcinia mangostana on inflammation caused by Propionibacterium acnes

Mullika Traidej Chomnawang [1], Suvimol Surassmo, Veena S Nukoolkarn, Wandee Gritsanapan

Affiliations + expand

PMID: 17644272 DOI: 10.1016/j.fitote.2007.02.019

Abstract

The present study was aimed to investigate the activity of Thai medicinal plants on inflammation caused by Propionibacterium acnes in terms of free radical scavenging and cytokine reducing properties. P. acnes have been recognized as pus-forming bacteria triggering an inflammation in acne. Antioxidant activity was determined by DPPH scavenging and NBT reduction assay. The result showed that Garcinia mangostana possessed the most significant antioxidant activity and reduced reactive oxygen species production. Houttuynia cordata, Eupatorium odoratum, and Senna alata had a moderate antioxidant effect. In addition, Garcinia mangostana extracts could reduce the TNF-alpha production as determined by ELISA. Garcinia mangostana was highly effective in scavenging free radicals and was able to suppress the production of pro-inflammatory cytokines. This study has identified the promising source of anti-inflammatory agent which could be useful in treatment of acne vulgaris.

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Dosagem e Modo de Usar

- Extrato seco (10%): 600 mg ao dia.

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Review > Phytother Res. 2016 Aug;30(8):1243-64. doi: 10.1002/ptr.5640. Epub 2016 May 23.

Effects of Turmeric (Curcuma longa) on Skin Health: A Systematic Review of the Clinical Evidence

Alexandra R Vaughn [1], Amy Branum [2], Raja K Sivamani [3]

Affiliations + expand

PMID: 27213821 DOI: 10.1002/ptr.5640

Abstract

Turmeric (Curcuma longa), a commonly used spice throughout the world, has been shown to exhibit antiinflammatory, antimicrobial, antioxidant, and anti-neoplastic properties. Growing evidence shows that an active component of turmeric, curcumin, may be used medically to treat a variety of dermatologic diseases. This systematic review was conducted to examine the evidence for the use of both topical and ingested turmeric/curcumin to modulate skin health and function. The PubMed and Embase databases were systematically searched for clinical studies involving humans that examined the relationship between products containing turmeric, curcumin, and skin health. A total of 234 articles were uncovered, and a total of 18 studies met inclusion criteria. Nine studies evaluated the effects of ingestion, eight studies evaluated the effects of topical, and one study evaluated the effects of both ingested and topical application of turmeric/curcumin. Skin conditions examined include acne, alopecia, atopic dermatitis, facial photoaging, oral lichen planus, pruritus, psoriasis, radiodermatitis, and vitiligo. Ten studies noted statistically significant improvement in skin disease severity in the turmeric/curcumin treatment groups compared with control groups. Overall, there is early evidence that turmeric/curcumin products and supplements, both oral and topical, may provide therapeutic benefits for skin health. However, currently published studies are limited and further studies will be essential to better evaluate efficacy and the mechanisms involved. Copyright © 2016 John Wiley & Sons, Ltd.

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Vitex agnus castus
A Systematic Review of Adverse Events

Claudia Daniele,[1] *Joanna Thompson Coon*,[2] *Max H. Pittler*[2] and *Edzard Ernst*[2]

1 Department of Pharmacology of Natural Substances and General Physiology, University of Rome 'La Sapienza', Rome, Italy
2 Complementary Medicine, Peninsula Medical School, Universities of Exeter and Plymouth, Exeter, UK

Abstract

Vitex agnus castus L. (VAC) [Verbenaceae] is a deciduous shrub that is native to Mediterranean Europe and Central Asia. Traditionally, VAC fruit extract has been used in the treatment of many female conditions, including menstrual disorders (amenorrhoea, dysmenorrhoea), premenstrual syndrome (PMS), corpus luteum insufficiency, hyperprolactinaemia, infertility, acne, menopause and disrupted lactation. The German Commission E has approved the use of VAC for irregularities of the menstrual cycle, premenstrual disturbances and mastodynia. Clinical reviews are available for the efficacy of VAC in PMS, cycle disorders, hyperprolactinaemia and mastalgia, but so far no systematic review has been published on adverse events or drug interactions associated with VAC. Therefore, this review was conducted to evaluate all the available human safety data of VAC monopreparations. Literature searches were conducted in six electronic databases, in references lists of all identified papers and in departmental files. Data from spontaneous reporting schemes of the WHO and national drug safety bodies were also included. Twelve manufacturers of VAC-containing preparations and five herbalist organisations were contacted for additional information. No language restrictions were imposed. Combination preparations including VAC or homeopathic preparations of VAC were excluded. Data extraction of key data from all articles reporting adverse events or interactions was performed independently by at least two reviewers, regardless of study design. Data from clinical trials, postmarketing surveillance studies, surveys, spontaneous reporting schemes, manufacturers and herbalist organisations indicate that the adverse events following VAC treatment are mild and reversible. The most frequent adverse events are nausea, headache, gastrointestinal disturbances, menstrual disorders, acne, pruritus and erythematous rash. No drug interactions were reported. Use of VAC should be avoided during pregnancy or lactation. Theoretically, VAC might also interfere with dopaminergic antagonists. Although further rigorous studies are needed to assess the safety of VAC, the data available seem to indicate that VAC is a safe herbal medicine.

COMUNIDADE Saúde Conectada

Licenciado para Lays Martins 074.232.516-40

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DOI: 10.1111/jocd.14345

ORIGINAL CONTRIBUTIONS

JCD Journal of Cosmetic Dermatology WILEY

Effects of a combination of botanical actives on skin health and antioxidant status in post-menopausal women: A randomized, double-blind, placebo-controlled clinical trial

Pakagamon Tumsutti MD[1] | Mart Maiprasert MD[1,2] | Pansak Sugkraroek MD[1,3] | Rungsima Wanitphakdeedecha MD[4] | Akkarach Bumrungpert PhD[1,2]

Abstract

Background: Skin aging is one of the most concerning issues during the post-menopausal period. Despite the promising effects of hormonal therapy, there is still concerned about the long-term outcomes from the treatment. Therefore, nutraceuticals that contain estrogenic and antioxidative effects have gained a lot of attention as an alternative therapy for slowing down skin age-related changes in women after menopause.

Objective: This study was aimed at evaluating the effects of a combination of nutraceuticals on skin health and antioxidant status in women after menopause.

Methods: Post-menopausal women aged 45–60 years old were enrolled and randomly allocated (n = 110) equally to either treatment or placebo group (n = 55 per group). The test product, a nutraceutical containing a blend of *Glycine max*, *Cimicifuga racemosa*, *Vitex agnus-castus*, and *Oenothera biennis* extracts, was administered over a 12-week period, with dermatological parameters evaluated at baseline, week 6, and week 12 of the study. Additionally, glutathione (GSH) and malondialdehyde (MDA) levels were detected at baseline and week 12 to evaluate the antioxidant status.

Results: At week 6, skin roughness was significantly improved in the treatment group (n = 50 completed), while at week 12, a significant improvement and large effect sizes observed in skin elasticity (Cohen's d = 1.56, [SD_{pooled} = 0.10]), roughness (d = 1.53, [0.67]), smoothness (d = −1.33, [34.65]), scaliness (d = −0.80 [0.095]), and wrinkles (d = −1.02 [13.68]) compared to placebo (n = 51 completed). Moreover, GSH was significantly increased (d = 1.54 [32.52]) whereas MDA was significantly decreased (d = −1.66, [0.66]) in the test group, compared to placebo. Blood biochemistry, along with vital signs, did not differ between groups, and no subjects reported any adverse throughout the trial.

Conclusion: These data indicate the supplementation with the formulated blend of four herbal extracts is supportive of skin health and antioxidant status in women of menopausal age.

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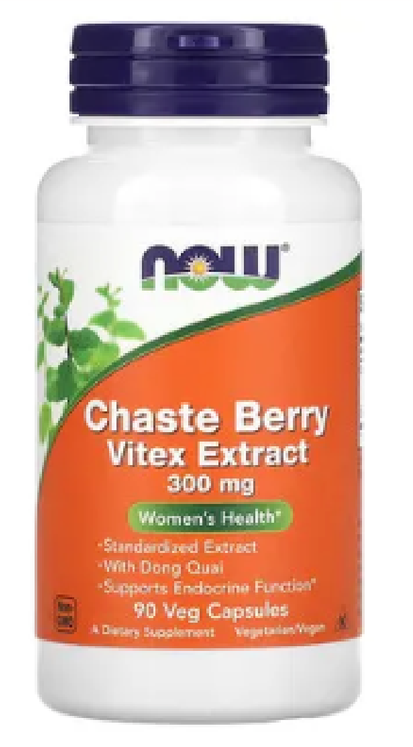

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Licenciado para Lays Martins 074.232.516-40

Review > Dermatol Ther. 2017 Sep;30(5). doi: 10.1111/dth.12481. Epub 2017 Feb 21.

The role of nicotinamide in acne treatment

Frances M Walocko [1], Ariel E Eber [2], Jonette E Keri [2 3], Mana A Al-Harbi [4], Keyvan Nouri [2]

Affiliations + expand

PMID: 28220628 DOI: 10.1111/dth.12481

Abstract

Safe and effective treatment options for acne vulgaris are needed to address side effects and increasing rates of antibiotic resistance from current treatments. Nicotinamide is a vitamin with potent anti-inflammatory properties that could offer a potential treatment option. We aim to summarize the relevant literature on the role of nicotinamide in acne vulgaris and discuss the next steps necessary to move this approach into clinical practice. We searched PubMed for clinical studies using nicotinamide for treatment of acne vulgaris. We summarized the 10 studies that met our search criteria. Six of eight studies using topical nicotinamide led to a significant reduction in acne compared with the patient's baseline or performed similarly to another standard-of-care acne treatment. Both studies using an oral supplement containing nicotinamide resulted in a significant reduction in acne compared with baseline. No major adverse side effects were noted. Our review suggests that topical and oral nicotinamide has an unclear effect on acne vulgaris due to the limited nature of the available literature. Additional studies are needed comparing nicotinamide to other first-line acne treatments and evaluating the efficacy and side effect profile of nicotinamide over an extended period of time.

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Indicações

• Suplementação.
• Tratamento e prevenção do déficit de vitamina B3 (pelagra).
• Tratamento tópico de acne leve a moderada.
• Tratamento oral com tetraciclina para controle de lesões de pênfigo

Posologia

A Vitamina B3, na forma ácida ou amida, é normalmente administrada na faixa de 20 a 100mg ao dia para o tratamento e prevenção dos estados de sua deficiência. A Nicotinamida tem sido utilizada em formulações tópicas para o trat- amento da acne, na concentração de 4%.

Licenciado para Lays Martins 074.232.516-40

Randomized Controlled Trial > Acta Derm Venereol. 2014 Sep;94(5):521-5.
doi: 10.2340/00015555-1802.

Effect of dietary supplementation with omega-3 fatty acid and gamma-linolenic acid on acne vulgaris: a randomised, double-blind, controlled trial

Jae Yoon Jung [1], Hyuck Hoon Kwon, Jong Soo Hong, Ji Young Yoon, Mi Sun Park, Mi Young Jang, Dae Hun Suh

Affiliations + expand

PMID: 24553997 DOI: 10.2340/00015555-1802

Free article

Abstract

This study was undertaken to evaluate the clinical efficacy, safety, and histological changes induced by dietary omega-3 fatty acid and γ-linoleic acid in acne vulgaris. A 10-week, randomised, controlled parallel dietary intervention study was performed in 45 participants with mild to moderate acne, which were allocated to either an omega-3 fatty acid group (2,000 mg of eicosapentaenoic acid and docosahexaenoic acid), a γ-linoleic acid group (borage oil containing 400 mg γ-linoleic acid), or a control group. After 10 weeks of omega-3 fatty acid or γ-linoleic acid supplementation, inflammatory and non-inflammatory acne lesions decreased significantly. Patient subjective assessment of improvement showed a similar result. Heamatoxylin & eosin staining of acne lesions demonstrated reductions in inflammation and immunohistochemical staining intensity for interleukin-8. No severe adverse effect was reported. This study shows for the first time that omega-3 fatty acid and γ-linoleic acid could be used as adjuvant treatments for acne patients.

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Review > J Cosmet Dermatol. 2021 Dec;20(12):3802-3807. doi: 10.1111/jocd.14057.
Epub 2021 Apr 15.

Vitamin D status and efficacy of vitamin D supplementation in acne patients: A systematic review and meta-analysis

Meng Wang [1], Yuan Zhou [1], Yan Yan [1]

Affiliations + expand

PMID: 33690970 DOI: 10.1111/jocd.14057

Abstract

Background: Acne vulgaris, a chronic inflammatory dermatosis, affects approximately 85% of adolescents and young adults.The level of vitamin D in acne patients and the effect on acne patients of a vitamin D supplementation have previously been documented; however, it remains to be disputed whether vitamin D deficiency is normal in acne patients.

Aims: This paper explores the relationship between vitamin D and acne levels and the advantageous implications of vitamin D supplements on acne.

Methods: From every database start date until December 2020, we checked the PubMed, EMBASE, MEDLINE and the Cochrane libraries. Case-controls or randomized controlled trial reporting on the serum 25(OH)D level and the efficacy of vitamin D supplement to acne patients were included.

Results: The serum 25(OH)D levels in acne patients is lower (standardized mean difference = -7.66 ng/ml; 95% confidence interval (CI) = -10.92 to -4.40). Articles also revealed the prospect of a therapeutic breakthrough through the supplementing of vitamin D for acne patients.

Conclusion: Our research offers a new guide in the field of acne prevention and health care.

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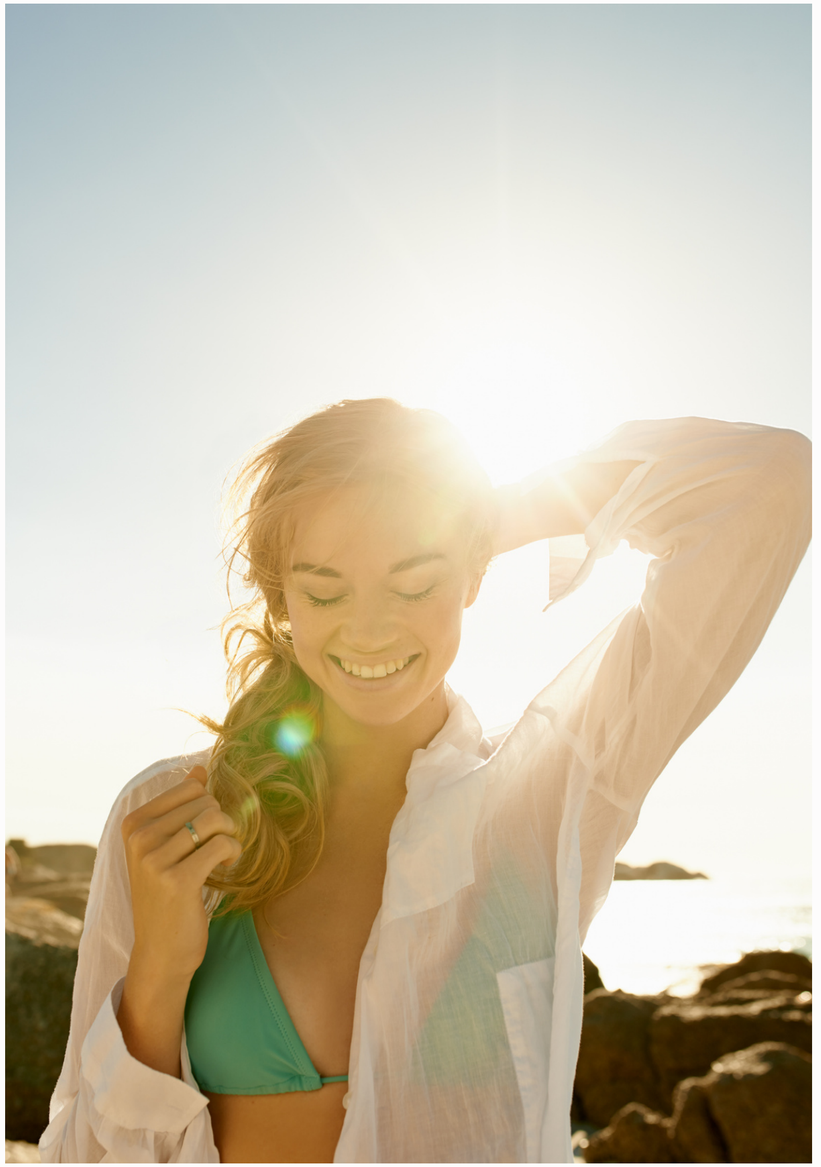

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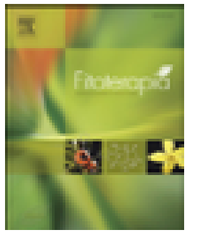

Review

A review of phytotherapy of acne vulgaris: Perspective of new pharmacological treatments

Hanieh Azimi, Mehrnaz Fallah-Tafti, Ali Asghar Khakshur, Mohammad Abdollahi *

Faculty of Pharmacy and Pharmaceutical Sciences Research Center, Tehran University of Medical Sciences, Tehran, Iran

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Review > Phytother Res. 2014 Aug;28(8):1137-52. doi: 10.1002/ptr.5125. Epub 2014 Feb 12.

Botanical and phytochemical therapy of acne: a systematic review

Whitney A Fisk [1], Hadar A Lev-Tov, Raja K Sivamani

Affiliations + expand

PMID: 25098271 DOI: 10.1002/ptr.5125

Licenciado para Lays Martins 074.232.516-40

Review > Infect Disord Drug Targets. 2021;21(3):328-338.
doi: 10.2174/1871526520666200622142710.

Miracles of Herbal Phytomedicines in Treatment of Skin Disorders: Natural Healthcare Perspective

Ashima Ahuja [1], Jitendra Gupta [1], Reena Gupta [1]

Affiliations + expand

PMID: 32568024 DOI: 10.2174/1871526520666200622142710

Abstract

Human skin is considered as the first line of defense and barrier against the majority of infections caused through the skin that affect humans. Healthy skin promotes a healthy body that can be achieved with the usage of modern, allopathic and natural remedies. Major skin ailments affecting humans are skin cancers, eczema, herpes infection, fungal infection, anti-aging, itching, insect bites, pemphigus vulgaris, trauma, psoriasis, athlete's foot infections, rashes, skin pigmentation, acne, major and minor wound infections that are slowly becoming a burden on health care. Skin infections can be treated from sources that originate from animals and plants. In spite of advancements in science and technology, the emergence of natural herbal remedies for managing skin disorders has become a pivotal and essential contributor in treating skin infections due to increased demand for herbals and their lower price, and continuous adverse effects of modern medicines. In the recent era, herbal extracts and their phytomedicines have made a vital contribution to human health care. Herbal products nowadays are considered as a single line of treatment for many diseases like Cancers, Diabetes, Cardiovascular and Brain disorders, thereby creating awareness regarding purity, efficacy and safety of herbal medicines for health care management. Many therapeutically active natural herbal resources like Aloe, Neem, Liquorice, Tulsi, Amla, Papaya, Ginger and Eucalyptus are potent and safe in the treatment of dermatological infections. This review article summarizes the significance of herbal plants for protecting, treating and minimizing skin infections through the utility of herbal pharmaceuticals like creams, decocted extracts, poultice, paste and lotions that aid in the treatment of skin infections and diseases at a relatively lower cost with lesser side effects as compared to modern and allopathic medicines.

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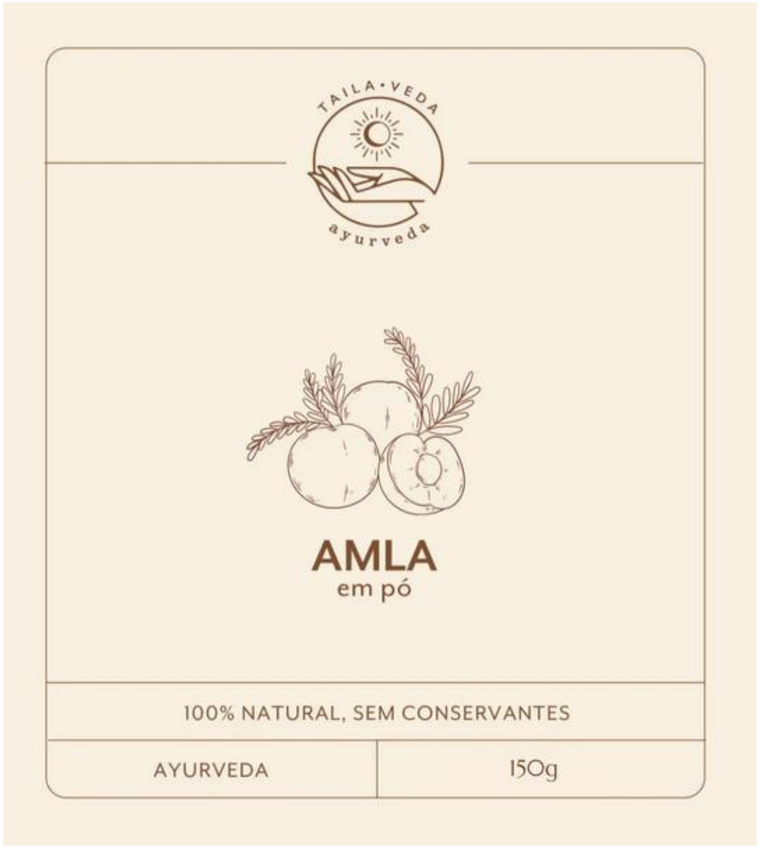

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Review > Arch Dermatol Res. 2020 Jan;312(1):5-23. doi: 10.1007/s00403-019-01968-z. Epub 2019 Aug 26.

Promising plant-derived secondary metabolites for treatment of acne vulgaris: a mechanistic review

Samaneh Soleymani [1], Mohammad Hosein Farzaei [2], Arman Zargaran [3], Somayeh Niknam [4], Roja Rahimi [5 6]

Affiliations + expand

PMID: 31448393 DOI: 10.1007/s00403-019-01968-z

Abstract

Acne vulgaris is the most common skin condition associated with inflammation of pilosebaceous unit. Since conventional therapies have not demonstrated desirable effectiveness and possess remarkable side effects, there is a growing interest in the use of herbal medicines for the management of acne vulgaris. In this study, plant-derived molecules investigated in acne vulgaris have been reviewed and their possible underlying mechanisms of action were discussed. For this purpose, different electronic databases including PubMed, Scopus, Cochrane library and Google Scholar were searched to obtain any in vitro, in vivo, or human studies evaluating the phytochemicals in the management of acne vulgaris. Data were collected from 1980 to 2018 (up to October). Most of the phytochemicals investigated in acne were from the category of polyphenols including resveratrol, myricitrin, schisandrin, terchebulin, alpha-mangotin, curcumin, ellagic acid and epigallocatechin 3-gallate. Moreover, alkaloids and terpenoids such as berberine, ursolic acid, lupeol were evaluated in acne vulgaris with less abundance. Various molecular mechanisms were involved in effects of phytochemicals including antioxidant (through down-regulation of H_2O_2, MDA, ROS and upregulation of SOD), anti-inflammatory (through reduction of proinflammatory cytokines, i.e., IL-1ß, IL-6, IL-8, TGF-β, TNF-α, NF-κB), immunomodulatory, antibacterial (against Propionibacterium acnes and Propionibacterium granulosum), antiandrogenic, reducing sebum production, and lipogenesis inhibitory activities. Therefore, phytochemicals seem to be a precious source for identifying new medicines for treatment of acne vulgaris; however, since most of studies are preclinical, further clinical studies are needed to achieve more conclusive and reliable results.

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Randomized Controlled Trial > J Dermatolog Treat. 2021 May;32(3):297-301.
doi: 10.1080/09546634.2019.1657792. Epub 2019 Oct 16.

Clinical efficacy of herbal extracts in treatment of mild to moderate acne vulgaris: an 8-week, double-blinded, randomized, controlled trial

Ji Hoon Yang [1 2], Eun Ju Hwang [3], Jungyoon Moon [4], Ji Young Yoon [2], Ji Won Kim [1], Sungjun Choi [1], Soo Ick Cho [1], Dae Hun Suh [1 2]

Affiliations + expand

PMID: 31424962 DOI: 10.1080/09546634.2019.1657792

Abstract

Background: Herbal extracts with fewer adverse effects can be an alternative to these drugs because they can target various molecular pathways of acne pathogenesis.

Objectives: To evaluate the clinical efficacy of herbal extracts (mangosteen, *Lithospermum officinale*, *Tribulus terrestris* L., *Houttuynia cordata* Thunb) for the treatment of mild to moderate acne vulgaris.

Methods: Sixty patients were randomized in a 1:1 ratio to receive blinded treatment with herbal extracts or vehicle for 8 weeks. Inflammatory and non-inflammatory acne lesion counts, Investigator's Global Assessment, patient's satisfaction and safety profiles were assessed. We also performed skin biopsy at baseline and week 8 to confirm immunological changes with immunohistochemistry staining.

Results: By the end of the study period, both inflammatory and non-inflammatory acne lesion counts were significantly decreased in herbal extracts group ($p < .05$). In immunohistochemistry staining, expressions of IL-1α, IL-8, and keratin 16 were significantly decreased in herbal extracts group compared to vehicle group from baseline to week 8. There was no serious adverse events in both groups.

Conclusions: This herbal extracts can be a new therapeutic option for patients with mild to moderate acne vulgaris who are reluctant to use drugs.

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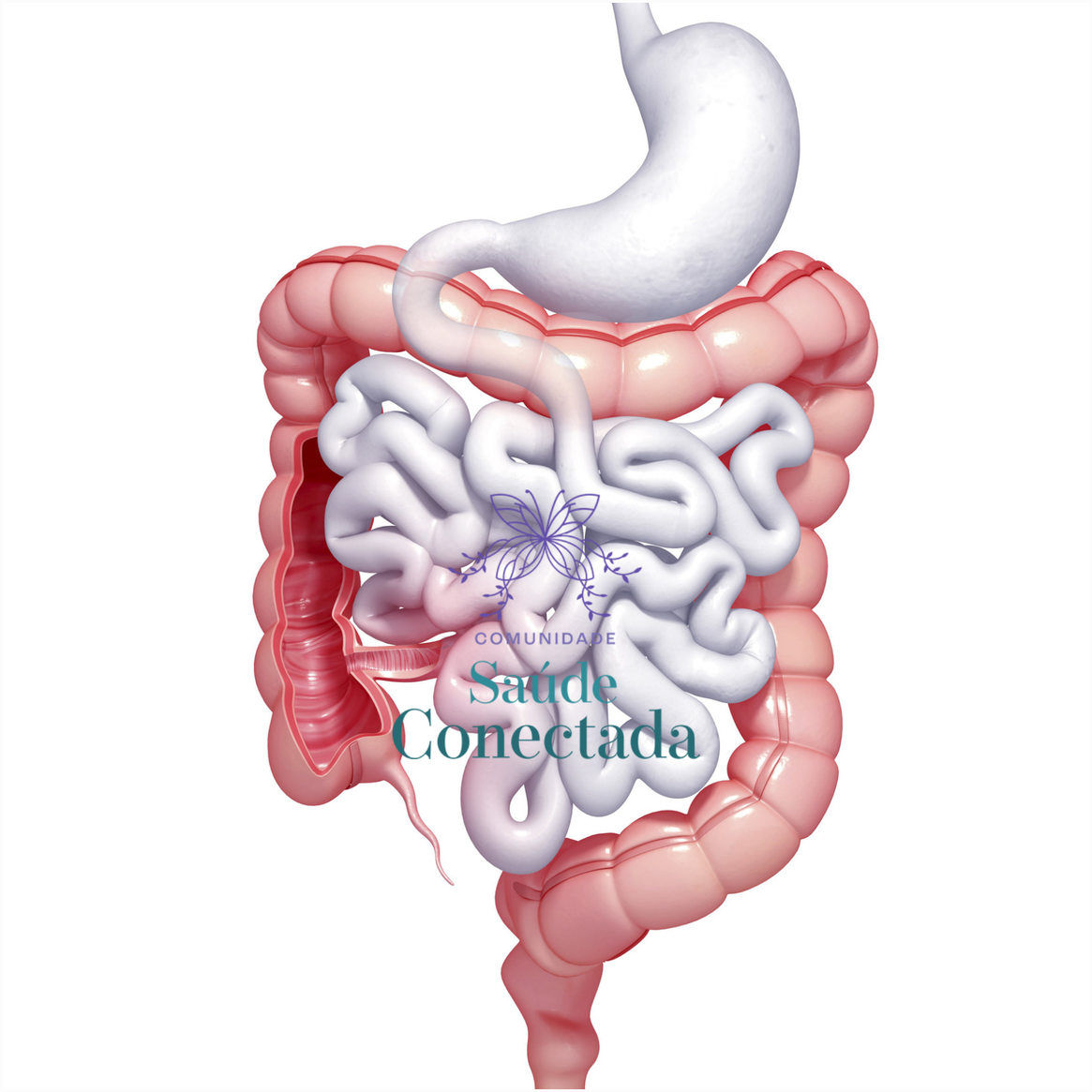

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Review > Br J Dermatol. 2020 Jan;182(1):39-46. doi: 10.1111/bjd.18088. Epub 2019 Jul 28.

Changing our microbiome: probiotics in dermatology

Y Yu [1], S Dunaway [1], J Champer [2], J Kim [3], A Alikhan [4]

Affiliations + expand

PMID: 31049923 DOI: 10.1111/bjd.18088

FULL TEXT LINKS

OXFORD ACADEMIC

ACTIONS

Cite

Collections

Comunidade Saúde Conectada

One study showed that consumption of Lactobacillus acidophilus, Lactobacillus delbrueckii bulgaricus, and B. bifidum was as effective as minocycline in the treatment of acne, with a 67% lesion reduction after twelve weeks and fewer side effects. A combination of this oral probiotic and minocycline had even greater effectiveness.

Another study showed a 30% reduction in inflammatory lesions after daily consumption of L. bulgaricus and S. thermophilus for twelve weeks.

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Review > Acta Dermatovenerol Croat. 2021 Nov;29(4):215-223.

Acne Supplementation: Probiotics, Vitamins, and Diet

Camilla Chello [1], Claudio Conforti, Caterina Dianzani

Affiliations + expand

PMID: 36896693

Abstract

Acne is an inflammatory disease of the pilo-sebaceous unit, which mainly affects young adolescents. The pathogenesis is multifactorial, as a combination of genetic predisposition, hormonal production, hyper-seborrhea, inflammation and overgrowth of C. acnes. Moreover, diet-induced hyperinsulinemia can lead to sebocyte stimulation, androgen production, and thus acne lesions. Pharmacological therapeutic approaches are varied and include topical and systemic antibiotics and, in severe cases, systemic retinoids, with several side-effects. However, increasing evidence has shown that adequate vitamin supplementation, use of probiotics, and proper nutrition with low carbohydrates and fats intake, can significantly contribute to the patient's clinical improvement. In this review, we describe the role of probiotics, vitamins, alimentation, antioxidants, UV exposure, and plants in acne disease.

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International Journal of
Molecular Sciences

MDPI

Review

Edible Plants and Their Influence on the Gut Microbiome and Acne

Ashley K. Clark [1], Kelly N. Haas [2] and Raja K. Sivamani [2,3,*]

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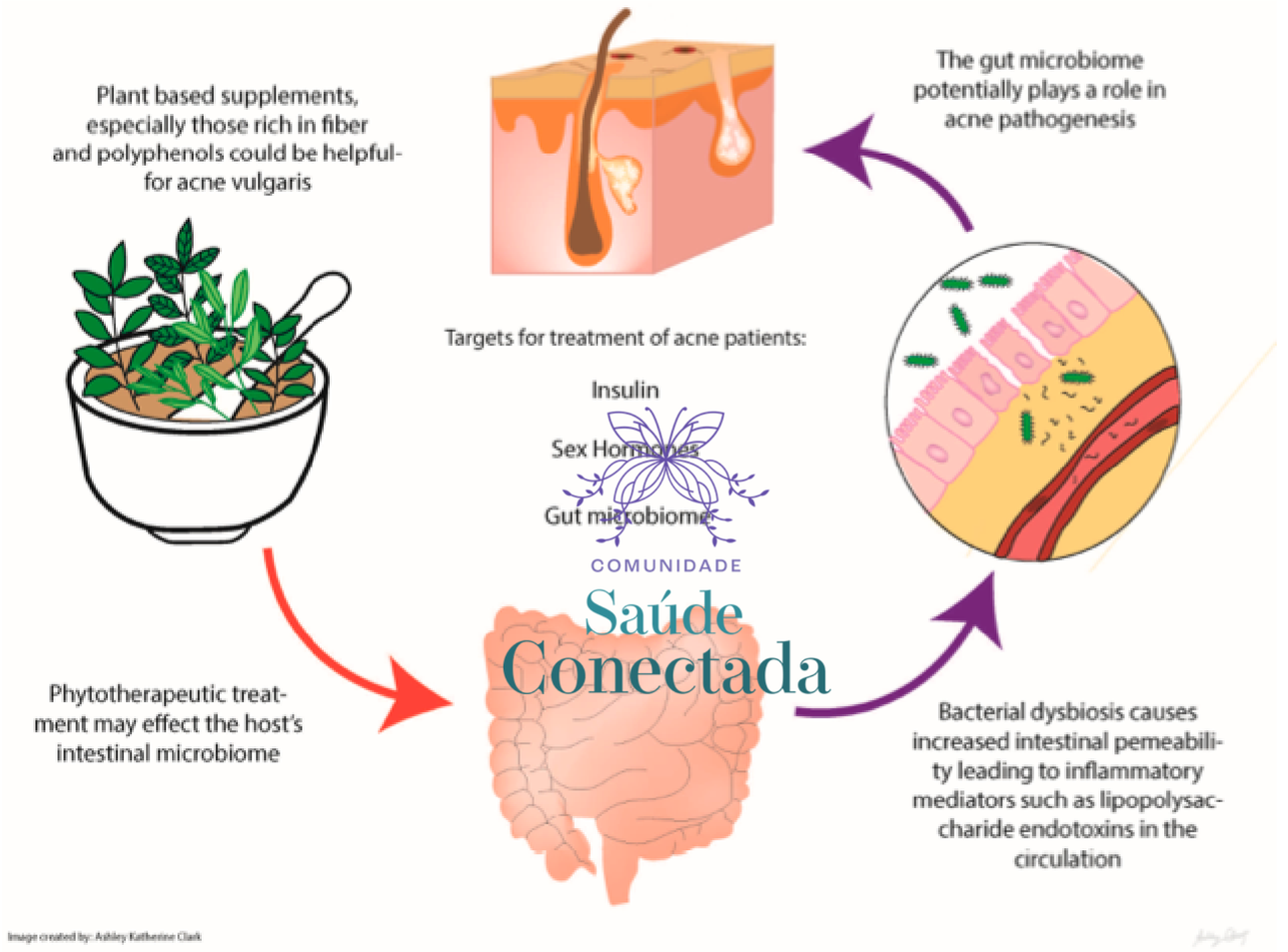

Figure 1. Edible plants and their influence on the gut microbiome and acne.

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Table 1. Summary of oral plant extracts for acne treatment.

Phytotherapeutic	Mechanisms	Study Type	Comparison	Number of Subjects	Outcomes	Reference
Ayurvedic plant extracts: *Aloe barbadensis, Azadirachta indica, Curcuma longa, Hemidesmus indicus, Terminalia chebula, Terminalia arjuna* and *Withania somnifera*	Antibacterial and anti-inflammatory activity *Azadirachta indica* = antipyretic and anthelmintic *Curcuma longa* (turmeric) and *Azadirachta indica* = anti-inflammatory effects by suppressing *Propionibacterium acnes*-induced reactive oxygen species and pro-inflammatory cytokines	In vitro Double-blind, placebo-controlled clinical trial in India	Placebo	53 mild to moderately severe acne patients 14–28 years old tested	The study found that the combined treatment of tablets and topical formulation of the plant extracts showed better results than the tablets alone, but the oral preparation was more efficacious than the topical alone	Lalla et al. (2001) [14]
Berberine	Relieve insulin resistance Antimicrobial against common skin microbes, like *Propionibacterium* acnes, *Staphylococcus* spp. and *Malassezia* spp. Antiproliferative effect on keratinocytes Decrease lipogenesis of sebaceous glands	Randomized controlled trial Hepatic cells in vitro In hamsters	Herbal supplement vs. control group taking minocycline	92 patients with acne vulgaris	No difference between the berberine and minocycline group; this suggests that herbal supplementation may be just as effective as the standard antibiotics without the drawbacks [15]	He et al. (2015) [15]; Slobodníková et al. (2004) [16]; Hu et al. (2016) [17]; Muller et al. (1995) [18]; Seki et al. (1993) [19]
Berry extract (most data on strawberries)	Polyphenol with unknown molecular mechanism	In vitro	None	None	Berry extract reduced glucose uptake by human intestinal epithelial cells	Kim et al. (2016) [20] Lu et al. (2016) [21]
Garcinia (α mangostin)	Antibacterial, attenuation of de novo lipid synthesis	In vitro Human	None	None	Improved insulin sensitivity and attenuated LPS-induced inflammation Topical application improved acne severity	Hayamizu et al. (2003) [22]; Pan-In et al. (2015) [23]
Green tea extract	Epigallocatechin-3-gallate (EGCG), the major polyphenol in green tea, has potent anticarcinogenic, anti-inflammatory and antimicrobial activities; EGCG can modulate several key pathological factors of acne, including hyperseborrhea, lipogenesis, inflammation and *P. acnes* overgrowth	Randomized, double-blind, placebo controlled clinical trial	1500 mg of decaffeinated green tea extract vs. placebo (cellulose)	80 25–45 year-old women with post-adolescent acne	Decreased acne lesions in postpubescent females with a trending decrease in fasting blood sugar	Lu et al. (2016) [21]
Gugulipid	Potent hypolipidemic agent; Antimicrobial, anthelmintic, anti-inflammatory, anti-arthritic and antioxidant properties	Randomized controlled trial	Tetracycline 500 mg vs. gugulipid 25 mg	Twenty patients with nodulocystic acne	Both produced a progressive reduction in the lesions; with tetracycline, the percentage reduction in the inflammatory lesions was 65.2% as compared to 68% with gugulipid ($p > 0.05$)	Thappa et al. (1994) [24]; Goyal et al. (2011) [25]

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Table 1. *Cont.*

Phytotherapeutic	Mechanisms	Study Type	Comparison	Number of Subjects	Outcomes	Reference
Sunder Vati (*Holarrhena antidysenterica, Emblica officinalis* and *Zingiber officinale*)	Unknown	An Indian double-blind placebo-controlled trial	Placebo	20	Treatment was associated with significant reduction ($p < 0.01$) in lesion count of approximately 60%; there was a significant reduction in the total number of inflammatory lesions within 2 weeks with further reduction at each observation period during the 6-week treatment	Paranjpe et al. (1995) [26]
Turmeric	Antimicrobial, anti-inflammatory and antidiabetic	In vitro	None	None	Several studies have shown growth inhibition of the common skin bacteria *Propionibacterium acnes*, *Staphylococcus epidermidis* and *Staphylococcus aureus* when curcumin is used topically	Liu et al (2013) [27]; Mun et al. (2013) [28]; Hegge et al. (2012) [29]
Vitex	Binds to estrogen receptors Acts on follicle-stimulating hormone and luteinizing hormone levels in the pituitary to increase progesterone levels Increase in estrogen level	In vitro	None	None	Commonly used in traditional Chinese medicine for menopausal symptoms, but their role as estrogen analogues also makes them promising for attenuation of acne	Allahtavakoli et al. (2015); Bedi et al. (2002); Ahangarpour et al. (2016)

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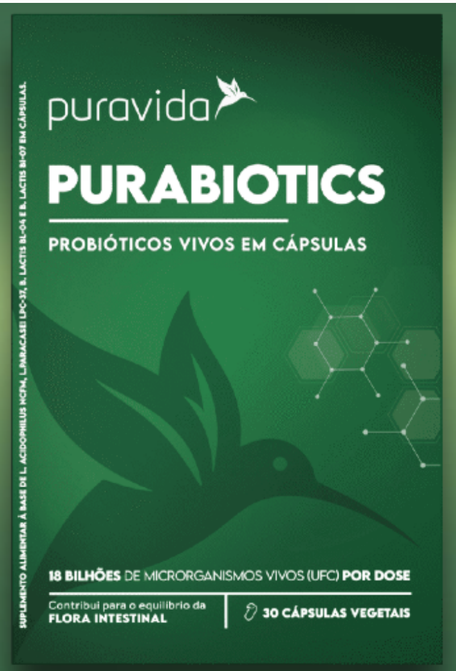
INGREDIENTES

Lactobacillus acidophilus NCFM (ATCC SD5221), Lactobacillus paracasei Lpc-37 (ATCC SD5275), Bifidobacterium animalis subsp lactis BI-04 (ATCC SD5219), Bifidobacterium animalis subsp lactis Bi-07 (ATCC SD5220), agente de massa celulose microcristalina, antiumectantes estearato de magnésio e dióxido de silício, cápsula (espessante hidroxipropilmetilcelulose).

NÃO CONTÉM GLÚTEN.



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INFORMAÇÃO NUTRICIONAL

Porção de 1,9 g (3 cápsulas)

Quantidade por porção		%VD
Lactobacillus acidophilus NCFM	$4,5x10^9$ UFC	*
Lactobacillus paracasei Lpc-37	$4,5x10^9$ UFC	*
Bifidobacterium animalis subsp lactis BI-04	$4,5x10^9$ UFC	*
Bifidobacterium animalis subsp lactis Bi-07	$4,5x10^9$ UFC	*

Não contém quantidades significativas de valor energético, proteínas, carboidratos, gorduras totais, gorduras saturadas, gorduras trans, fibra alimentar e sódio. *VD não estabelecido.

SUGESTÃO DE USO

ADULTOS (MAIORES DE 19 ANOS), **INGERIR 3 CÁPSULAS AO DIA.**

USO NOTURNO, ANTES DE DORMIR

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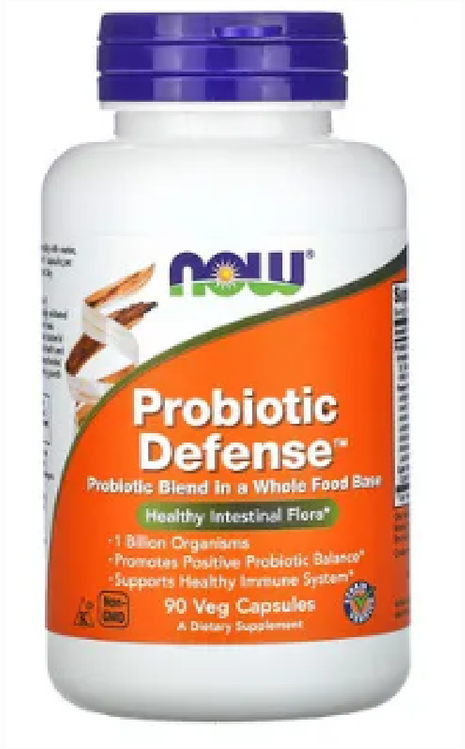

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SEGUNDO PASSO

Como usar ?

1. Escolher DOIS suplementos para ACNE
2. Pode acrescentar esses dois aos escolhidos de outras imersão
3. Usar na dose indicada do fabricante
4. Utilizar por até 3 meses e rotacionar caso seja ainda necessário.

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TERCEIRA PASSO

Mascaras naturais

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Open Access Full Text Article

ORIGINAL RESEARCH

Treatment of acne with a combination of propolis, tea tree oil, and *Aloe vera* compared to erythromycin cream: two double-blind investigations

Introduction: Antibiotics that suppress *Propionibacterium acnes* are the standard treatment for acne but are becoming less effective, due to the appearance of antibiotic-resistant strains. Many plants are known to have innate antimicrobial action and can be used as alternatives to antibiotics; thus, it is necessary to prove their effectiveness in vivo. This study aimed to evaluate the anti-acne efficacy of a new cream based on three natural extracts, comparing it to erythromycin cream and placebo.

Patients and methods: Sixty patients with mild to moderate acne vulgaris were randomly divided into three groups: treated with cream containing 20% propolis, 3% "tea tree oil", and 10% "*Aloe vera*" (PTAC) (n=20); or with 3 % erythromycin cream (ERC) (n=20); or with placebo (n=20). At baseline, after 15 and 30 days, investigators evaluated response to treatment by counting acne lesions through noninvasive measurements and macro-photography.

Results: All the clinical and instrumental values studied were statistically different from placebo except for sebometry, pHmetry, and erythema index values, measured on healthy skin. Unlike in the placebo group, papular and scar lesions showed high erythema reduction after 15 and 30 days of PTAC and ERC application.

Conclusion: The PTAC formulation was better than ERC in reducing erythema scars, acne severity index, and total lesion count.

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Máscara 1

- 1 col. de chá de curcuma pó
- 1/4 suco de limão
- 1/2 col. de chá de mel
- Misturar tudo e aplicar na região afetada pela manhã. Deixar agir de 15 a 20 min e lavar bem com água morna

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Máscara 2

- 1 col. de sopa de óleo de gergelim prensado a frio.
- 1 col. de chá de cominho em pó
- Misturar tudo e aplicar na região afetada pela manhã. Deixar agir de 15 a 20 min e lavar bem com água morna

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Máscara 3

- 1 col. de sopa de água
- 1 col. de chá de noz moscada, cúrcuma em pó e pimenta do reino moída
- Misturar tudo e aplicar na região afetada pela mama. Deixar agir de 15 a 20 min e lavar bem com água morna

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Máscara 4

- O mamão é um ótimo remédio para acne e espinhas. Lave o rosto, seque e aplique pasta de mamão (com água ou água de rosas) no rosto. Deixe a máscara por 20 minutos e enxágue com água morna. Repita diariamente.

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Máscara 5

- Óleo para rosto: 1 gota em 1 colher de sopa de carreador

Aplicar a mistura de pasta de sândalo, cúrcuma com água de rosas na pele afetada todos os dias. Deixe agir por 30 minutos e lave com água fria.

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Máscara 6
Use a cada 15 dias

- Uma colher de chá de açafrão da terra
- Duas colheres de chá de farinha de grão-de-bico
- leite de amêndoas ou leite de coco ou água ou leite de vaca suficiente para misturar os pós e fazer uma pasta

Misturar os ingredientes e aplicar logo em seguida na pele. Não guardar a mistura pronta para usar em outros momentos.

- Se a sua pele é muito seca recomendo que você troque a farinha de grão-de-bico pela farinha de aveia e misture um pouco de óleo de coco.
- Se sua pele já é um pouco mais oleosa não precisa colocar o óleo de coco e mantenha a mistura com a farinha de grão-de-bico.

- Se você não encontrar farinha de grão-de-bico pra vender onde você mora, você mesmo pode preparar. Pegue o grão de bico e deixe de molho cerca de 8 horas na água e depois escorra completamente essa água. coloque numa forma e leve ao forno pra secar por cerca de 40 minutos. retire do forno e deixe esfriar e bata no liquidificador bem potente ou num processador até formar uma farinha, passar numa peneira para deixar a farinha bem fininha e guardar em recipiente de vidro bem vedado.

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Óleos essenciais

Sândalo
Alecrim
Camomila
Limão siciliano

Accepted Manuscript

Aromatherapy, Botanicals, and Essential Oils in Acne

Warren Winkelman

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Ciclo das sementes

Abóbora

Linhaça

Gergelim

Girassol

O ciclo das sementes consiste no rodízio de quatro sementes – linhaça, abóbora, girassol e gergelim – ao longo do mês de acordo com a sua própria variação hormonal.

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As sementes devem ser consumidas torradas ou deixadas de molho, em qualquer momento do dia. Indico que a linhaça seja consumida batida em sucos ou na forma de farinha, para melhor absorção das lignanas.
Para quem não está menstruando, pode guiar o ciclo pelas fases da lua. Inicie a primeira fase com a lua nova e a segunda fase com a lua cheia.

Por quanto tempo fazer ?

O indicado é fazer por pelo menos 3 ciclos para observar resultados consistentes, apesar de muitas mulheres relatarem melhora dos sintomas antes disso.
Não há contra indicações em fazer uso contínuo. Em alguns casos alguns ajustes precisam ser feitos, como aumentar a quantidade de uma semente e reduzir de outra. Só recomendo pausa durante a gestação.
As contra indicações são somente para aquelas que têm alergia à alguma das sementes ou não se sentem bem ao consumí-las. Nesse casos, alguns ajustes podem ser necessários.

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Exemplo de dieta - 2 semanas

Este plano de dieta para acne elimina o glúten, todas as gorduras animais, carboidratos simples e açúcar, óleos e cacau, pois são as causas comuns da acne. É claro que nem todos eles causarão acne, mas é um bom começo para desintoxicar seu corpo e curar sua acne e, em seguida, introduzir os grupos de alimentos de volta em sua dieta.

Sugiro que faça a dieta para acne por 2-3 semanas para realmente ver os resultados em sua pele. Em seguida, traga de volta o glúten à sua dieta, mas com moderação (e apenas trigo integral orgânico ou espelta) e veja como você reage.

Após a melhora em 1 ou 2 semanas, traga outro grupo de alimentos para sua dieta, como cacau em pó (sem leite ou açúcar) ou óleos e veja como você reage.

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Muitas vezes, a quantidade que você come determinará se você terá surtos ou não.

Coisas que você deve sempre deixar de fora de nossa dieta são gordura animal em excesso, carboidratos simples, doces e fast food, pois eles causarão mais danos à sua saúde do que apenas acne.

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Almoço ou jantar

Bowl

- Arroz
- Lentilhas
- Brócolis
- Espinafre
- Cenouras
- Salsão
- Abacate
- 1/2 limão espremido
- Manjericão ou coentro (fresco)
- Açafrão e pimenta preta

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Almoço ou jantar

Bowl

- Batatas
- Pimentão vermelho
- Grão de bico
- Cebola
- Alho
- Pasta de tomate
- Pimenta em pó
- Açafrão e pimenta preta
- Curry em pó

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Almoço ou jantar

Bowl

- Pimentão verde
- Tomate picados
- Batata doce
- Cebola
- Alho
- Feijão
- Cominho
- Páprica em pó
- Pimenta
- Orégano
- Cobertura opcional: abacate, coentro

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Almoço ou jantar

Sopa de lentilha

- Lentilhas
- Alho
- Cenouras
- Batatas
- Gengibre fresco
- Cebola
- Salsão
- Couve-flor picada

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Almoço ou jantar

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Lanche

Smothie

- 2 xícaras de sua fruta favorita
- Leite vegetal
- Nozes e sementes
- Espinafre fresco

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Chás e água

- Certifique-se de beber muita água durante o dia para ajudar a curar seu intestino e hidratar a pele.
- Chás anti-inflamatórios e para o fígado.

Kit Intestino C/ 4 - By Dra. Pri Antunes

Kit Desinflamação C/ 4 - By Dra. Pri Antunes

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Suplementação

- Curcumina
- Ômega 3

Onde comprar?

- Site das marcas
- Mercado Livre (ver pontuação do vendedor)
- Amazon
- Iherb.com

Como tomar ?
Uma dose pela manhã, ou você pode dividir manhã e tarde

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